



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : علا جمال صابر محمد
المحافظة : البحيرة
الحى / المدينة : ابوالمطامير
رقم البطاقة : 29408072600248 رقم التليفون : 01148548091
العنوان : ابو المطامير
العمر (تاريخ الميلاد) : 07-08-1994 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: علا جمال صابر محمد
National ID: 29408072600248



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين


 وزارة الداخلية
 بطاقة تحقيق الشخصية

علا
 جمال صابر محمد
 قرية شرق الدواجن لك ٥٩
 مركز أبوالمظاير - البحيرة

٢٩٤٠٨٠٧٣٦٠٠٢٤٨

JE4079708



٢٠٢١/١١ ٢٩٤٠٨٠٧٣٦٠٠٢٤٨
 حاصلة على دبلوم تجارة
 متروجة
 أنثى
 ضاحي جلال محمد أحمد
 ٢٠٢٨/١١/٣٤
 المنطقة سارية حتى




 جمهورية مصر العربية وزارة الصحة والسكان 07/29/2026		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية	
MDT FORM			
Name: Oia Gamal Saber *		MRN: 3206	
Age: 32 Y		ID: 29408072600248	
Menopausal Sta: prem		Phone: * 01146548091	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender:		Reason Of MDT:	
CO Morbidity:		very good subjective response but no clear comp bec no cts done prior to st ge	
Surgical History: * none		ECOG Performance Status 0 1 2 3 4	

Clinical Data:

BX:

ER:

PR:

HER2:

K167:

History:

PREM MBC TO LUNG, MED LN BONE LEPTOMENINGES IDC 2 ER8/8 PR6/8
 HER2-VE. HISTORY LEFT BREAST CANCER MET TO LUNG MED LN BONE
 ST ZOLADEX AI KISQALI FROM 2022 TO 2024 DP AFINITOR AROMASIN DP
 TILLS/2025 DP LEPTOMENINGES MET REC WCI ST XELODA TILL 12/2025
 DP ST TAXOTERE x3 cycles 5/2026

then developed severe dyspnea newly developed rt pleural effusion
 so st gemzar carboplatin without baseline cts
 pt rec 4 cycles with very good respnse (no need of tapping improvement of
 dyspnea)

cts 16/7/2026 comp to 10/2026 before st gemzar 5/2026 by 8 m pt st
 after newly developed pleural effusion

pd of pulmonary nodules newly developed bilobar hft for met
 pt will bring cts done in er and before tapping

Sonography:

Biomarker/Molecular

Meta Workup:

Recommendations:

MDT Outcome & Plan

No available films for comparison, in view of subjective response, consider
 returning 2 more cycles with short term CT scan assessment.



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
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د. شيماء لطفى
 21/7/2026

د. نادية حجازي
 29/7/2026

Patient's Name: Ala Jamal Saber Mohammed
Date: 2026-07-16
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION
REVEALED:**

**Compared to the previous CT scan performed on 19-10-2025, the
current study revealed:**

I- CHEST:

- Progressive course regarding the size and number of the previous noted multiple bilateral pulmonary nodules the largest is seen in the lateral segment of the left lower lobe measuring 26 x 21 mm.
- Newly developed encysted pleural effusion seen in the right upper and middle lobes
- No hilar or mediastinal masses or lymph node enlargement.
- Still noted left mild pleural effusion.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles.
- Newly developed few bilobar hepatic hypodense focal lesions the largest is seen in the left lobe segment II measuring 22 x 21 mm showed heterogenous mainly peripheral enhancement
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.

Patient's Name: Ala Jamal Saber Mohammed
Date: 2026-07-16
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- No ascites.
- Still noted right 10 rib sclerotic lesion with extra-osseous component.
- Newly developed multiple sclerotic lesions seen in bilateral iliac bones and dorsal and lumbar vertebral bodies

Opinion:

- ❖ Progressive course since the last study.
- ❖ Multiple hepatic, pulmonary and osseous metastatic lesions.

Dr.Assem Ahmed



Patient Name : علا جمال صابر محمد
Sex :
Age : 27
Clinical Data: Lt. breast mass, biopsy.
Department : جراحه - ج
Pathology No : 6562/22
Hospital No. 9098/22

Referring Dr. :

Electronically Authorized by: BASSANT ABDELMONIEM

Gross :

One referred paraffin block coded 647494.

Histologic :

Cores show invasive carcinoma formed of irregular solid groups of malignant ductal epithelial cells with focal ductal differentiation. Anaplasia and mitosis are moderate. No associated DCIS. Surrounding stroma shows mild fibrosis. No evidence of lympho-vascular permeation. ER shows moderate positive staining in about 40% of tumor cells (6/8). PR shows strong positive staining in about 70 % of tumor cells (8/8). Her-2neu is negative score 0.

Diagnosis Breast, Left, Referred slide: Invasive duct carcinoma, Grade 2.

Category Malignant primary site (Single tumour)

Pathologists :

Dr.

Mai Gad Elsayed Ibrahim

Dr.

Dr.

BASSANT ABDELMONIEM

REPORT WAS SIGNED ON 05/06/2022

Verbal Revision

BASSANT ABDELMONIEM

Typist

sana'a abdel aziz ismael

Date of Issue : 06/07/2022

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

-Patient-Name : علا جمال حناوير

Age : 27 Y

Referred By Prof. Dr. : محمد صبرى القاضى

Clinical Diagnosis : Left breast mass and pathological axillary lymph nodes

Nature of Specimen : Block

Receiving Date : 2022-05-16

Analysis ID : 647495

Gender : Female

Patient ID : 502309

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.
The sections were treated against:

- Estrogen receptors.
- Progesterone receptors.
- Her 2.
- Ki67.

Results :

60% of tumor cells show MODERATELY POSITIVE Staining for Estrogen receptors.
90% of tumor cells show STRONGLY POSITIVE Staining for progesterone receptors.
Sections treated for HER2 revealed NEGATIVE CELL MEMBRANE STAINING.
40% of tumour cells are positive for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE:
POSITIVE FOR ESTROGEN RECEPTORS (SCORE 5 / 8).
POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 8 / 8).
NEGATIVE FOR HER 2 PROTEIN OVEREXPRESSION, SCORE 0.
THE PROLIFERATION INDEX OF Ki67 IS 40% OF THE TUMOUR.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

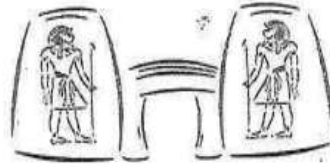
Shady Elia

Elia anis

This report is electronically signed, any abrasion or modification would render it void.

صناعة رمسيس - بلكة أ - ميدان رمسيس - الدور السادس و السابع
تلفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٢٢٥٣

الكرنك للأشعة والتحاليل Karnak Scan & Lab



Patient Name: Mrs. Ola Gamal Saber
Date: 15/05/ 2022
Ref. By: Prof. Dr. Mahmoud Mohamed Abo El Yazed

BILATERAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASONOGRAPHY

REVEALED-

CLINICAL SUMMARY:

- Left breast lump sensation of two months duration with left nipple retraction.

FINDINGS: -

- Normal skin thickness and contour with normal nipple/ areola complex and subcutaneous tissues.
- Predominant fibro-glandular element on both sides (dense breast).
- The left breast, at the LOQ and UOQ show a dense speculated mass lesion with surrounding small rounded lesion and rather central skin thickening (6 mm). In ultrasound, several, rather well defined soft tissue mass lesions, with irregular/ speculated outlines are noted at the outer aspect of the breast, along 2 and 4 O'clock orientation, the largest two measure 27x20.7x19.7 mm and 47.3x25x30.6 mm (with ductal extension), they show increased internal vascularity. The other smaller lesions are seen at the UOQ, measuring 9.5x 9 mm, 7.6x5.8 mm and 9.5x 6 mm.
- The right breast shows two oval well defined mass lesion, with regular outline along 9O'clock and 3 O'clock orientations, measuring 11.3x5.6 mm and 11.4x5.2 mm
- No suspicious solid or cystic lesions noted on the right side.
- No significant retro areolar ducts on either side.
- No suspicious micro-calcifications seen on the right side.
- Left axillary several pathological lymph nodes (cortical thickening =8.3 mm), also showing lymph node with lost fatty hilum, irregular outline (extracapsular extension). Few infracalvicular pathological lymph nodes are also noted, measuring 15.7x7.6 mm and 12.8x8 mm
- Right axillary non-specific lymph nodes with preserved fatty hila.

OPINION: -

- Left breast outer aspect several malignant looking soft tissue mass lesions with several pathological left axillary and left infraclavicular lymph nodes as described (BIRADS 5), for trucut biopsy and histopathology assessment.
- Right breast two small mass lesions of favorable criteria as described (e.g fibroadenomata) (BIRADS 3).

Dr. Mariana Fikry, MSc



أ.د. أحمد جمال الدري
أ.د. محمد أبو الهدى درويش
أ.د. عمر حسين عمر
أ.د. وهيد حسين طنطاوي
أ.د. ياسر عبد العظيم عباس
أ.د. أحمد محمد منيب
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

PATIENT NAME: OLA GAMAL SABER	PATIENT ID: 689923
REF. PHY.: MOHAMED SABRY EL QADY	STUDY DATE: 17/07/2025

MRI OF THE BRAIN:-

The study was performed on a high field MR unit.

TECHNIQUE:-

- Axial T1, FLAIR & T2 WIs
- Sagittal T1 WIs
- Axial D WIs
- Post Gd-DTPA stud

FINDINGS:

This is a 30-year-old with history of breast cancer and the aim of the study is to assess for any intracranial metastatic deposits.

- No recent infarction on D WIs.
- Normal MR signal intensity of the brain parenchyma with no focal lesions.
- Normal site, size and shape of the ventricular system.
- No shift of midline structures.
- No fresh intracerebral haematoma or extra-axial collection.
- Normal MR of the sella turcica and craniocervical region.
- No MR evidence of osseous metastatic deposits.
- The post contrast series showed minimally accentuated leptomeningeal enhancement infra and supratentorially, while to a lesser extent slightly pachymeningeal enhancement regarding the tentorial leaflets.

OPINION:-

- No recent infarcts on D WIs.
- No MR evidence of parenchyma metastatic deposits, however, there are MR data suggesting leptomeningeal and to a lesser extent pachymeningeal metastatic disease due to the breast cancer, yet this has to be correlated with clinical and laboratory data because there is some degree of overlap with inflammatory infectious process and the whole case is for follow up as appropriate.

MUCH OBLIGED

PROF.DR. YASSER ABD ELAZIM
M.S.

Patient Name: Ola Gamal Saber	Date : 19/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 31 Year(s)
Medical ID: 2583/2022	Pat. ID: 26434

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS

REVEALED: -

Reported case of left breast carcinoma after correlation with previous PET-CT report only, dated 28/04/2025 the current study shows the following points:

Chest:

- Multiple left breast masses , the largest seen in retroareolar region measuring 2.7x2.2cm with retracted nipple . (PET-CT 2.7x2)
- Further progression of the enlarged left axillary lymph nodes being 1.2x1cm in today's study.
- Progressive course regarding size and number of the forenoted multiple bilateral pulmonary nodules, the largest is noted at upper lobe of left lower lung lobe measuring 1.4x1.3 cm. (12--)
- Multiple enlarged hilar and peritracheal lymph nodes , the largest measures 1.3x1cm . (1.2)
- Mild left pleural effusion .
- Average cardiac size. No pericardial effusion detected.
- Normal appearance of the thoracic aorta and the great vessels.

1. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

2. The scanned skeleton:

- Sclerotic bony lesion is seen involving lateral aspect of right 10th rib with evidence of soft tissue component .
- A sclerotic lesion is seen in left pedicle of L5 vertebra .
- No vertebral structural collapse .

OPINION:

- **Progressive course regarding ;**
 - **Left breast malignant mass lesion and ipsilateral axillary lymph nodes as described.**
 - **Bilateral scattered pulmonary nodules regarding number & size and left pleural effusion .**
- **Stationary course regarding 10th rib sclerotic lesion with soft tissue component and L5 sclerotic lesion .**
- **No definite hepatic deposits.**

Signature Reeham.Khader
22/10/2025 1:23 AM